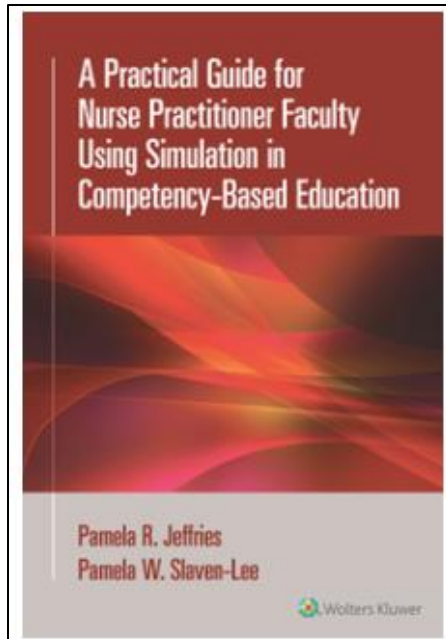


ATTAINMENT OF COMPETENCY THROUGH SIMULATION THE ACTS MODEL



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CONTINUING EDUCATION STATEMENT

Vanderbilt University School of Nursing is accredited as a provider of nursing continuing professional development by the American Nurses Credentialing Center's Commission on Accreditation.



This activity will award 1 contact hour to participants who attend the event and submit the required program evaluation. No partial attendance credit will be given.



Today's presenter has no financial relationships to disclose.

CONTEXT

- Belief that INACSL Standards™ didn't align well with experiential learning in NP education
- Lack of clarity in how to use simulation to meet competencies
- Perceived complexities of the AACN New Essentials (2021)
- Proliferation of experiential learning/simulation with pandemic
- Need for competency among educators in the use of simulation pedagogy
- Demand for a systematic, reproducible, standardized approach to providing opportunities for students to work toward competency
 - Facilitates attainment of learning outcomes
 - Provides opportunities for progress toward competencies

International Nursing Association for Clinical Simulation and Learning (INACSL) Healthcare Simulation Standards of Best Practice™ (HSSOBP)



OUTCOMES

1. Describe how the INACSL SHHOBP™ (2021) may be systematically applied to the Essentials: Core Competencies for Professional Nursing Education (AACN, 2021).
2. Describe how simulation experiences can allow for the partial attainment of the Level 2 sub-competencies required for advanced level nurse practitioner (NP) education.
3. Apply a framework for the design of clinical case scenarios in NP education, with emphasis on best practices in simulation in the attainment of a Level 2 sub-competency

Objective Structured Clinical Examination (OSCE)



ONWARD & UPWARD INASCL HSSOBP™ 2021

- Professional Development (NEW)
- Prebriefing: Preparation and Briefing (NEW)
- **Simulation Design - 11 Criterion**
- Facilitation
- The Debriefing Process
- Operations
- Outcomes and Objectives
- Professional Integrity
- Sim-Enhanced IPE
- Evaluation of Learning and Performance
- Simulation Glossary



COMPETENCY-BASED EDUCATION & SIMULATION

- **Assumptions**

- Students will demonstrate acquisition of essential competencies
- Simulation objectives are directly related to program outcomes
- Shift from **outcome** to **progress/process** toward attainment of competency
- Assessment and evaluation calls for identification of **progression indicators**
- Competence as “dynamic, developing, or receding over time, and as grounded in the environment of practice or learning.”
(Frank, et al. 2010, p641).



AACN NEW ESSENTIALS: DOMAINS & CONCEPTS

DOMAINS

1. Knowledge for Nursing Practice
2. Person-Centered Care
3. Population Health
4. Scholarship for Nursing Discipline
5. Quality and Safety
6. Interprofessional Partnerships
7. Systems-Based Practice
8. Informatics and Healthcare Technologies
9. Professionalism
10. Personal, Professional, and Leadership Development

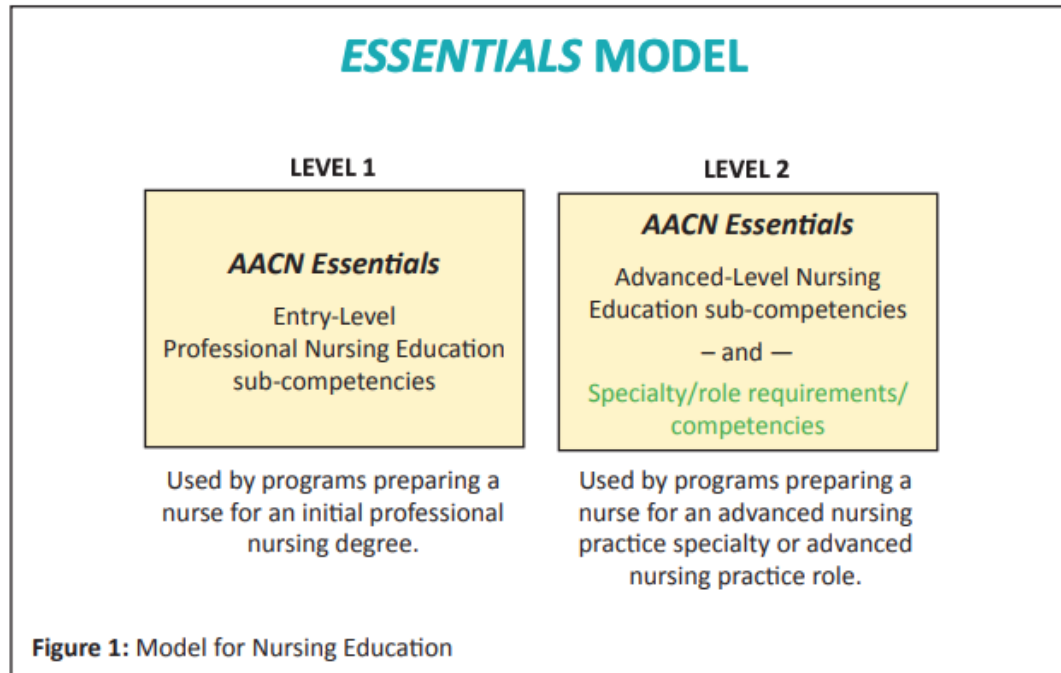
CONCEPTS FOR NURSING PRACTICE

1. Clinical Judgement
2. Communication
3. Compassionate Care
4. Diversity, Equity, and Inclusion
5. Ethics
6. Evidence-Based Practice
7. Health Policy
8. Social Determinants of Health



**DIFFERENT THAN CONTEXTUAL
CONCEPT**

COMPETENCIES & SUBCOMPETENCIES



DOMAIN 1: KNOWLEDGE FOR NURSING PRACTICE

Entry-Level Professional Nursing Education	Advanced-Level Nursing Education
1.1 Demonstrate an understanding of the discipline of nursing's distinct perspective and where shared perspectives exist with other disciplines	
1.1a Identify concepts, derived from theories from nursing and other disciplines, which distinguish the practice of nursing.	1.1e Translate evidence from nursing science as well as other sciences into practice.
1.1b Apply knowledge of nursing science that develops a foundation for nursing practice.	1.1f Demonstrate the application of nursing science to practice.
1.1c Understand the historical foundation of nursing as the relationship developed between the individual and nurse.	1.1g Integrate an understanding of nursing history in advancing nursing's influence in health care.
1.1d Articulate nursing's distinct perspective to practice.	
1.2 Apply theory and research-based knowledge from nursing, the arts, humanities, and other sciences.	
1.2a Apply or employ knowledge from nursing science as well as the natural, physical, and social sciences to build an understanding of the human experience and nursing practice.	1.2f Synthesize knowledge from nursing and other disciplines to inform education, practice, and research.

AACN COMPETENCY ASSESSMENT FRAMEWORK

ASSESSMENT FRAMEWORK

In summary, the *Essentials* Competency Assessment Framework includes four components which together create a platform for assessing student learning and readiness for professional nursing practice.

Competencies



For professional nursing practice which include knowledge, skills, values, and attitudes that cross all professional nursing practice.

Sub-Competencies



Encompass the behaviors or behavioral representations of an important aspect of a competency. Learner attainment of competency at the entry- or advanced-level is documented by demonstration and observation of the sub-competencies.

Progression Indicators



Defined as descriptive behaviors that show progression to competency and attainment of each sub-competency. Progression indicators describe specific behaviors that an assessor would expect to observe and provide markers of sub-competency development along the trajectory from developing competency to competent.

Nursing Practice Scenarios



Learning experiences which include clusters of related competencies and sub-competencies for both formative and summative assessment of competency achievement.

Updated November 2023



THE ESSENTIALS

For more tools related to the AACN *Essentials*, strategies for engaging practice partners, and approaches to moving to CBE, visit:

www.aacnnursing.org/Essentials



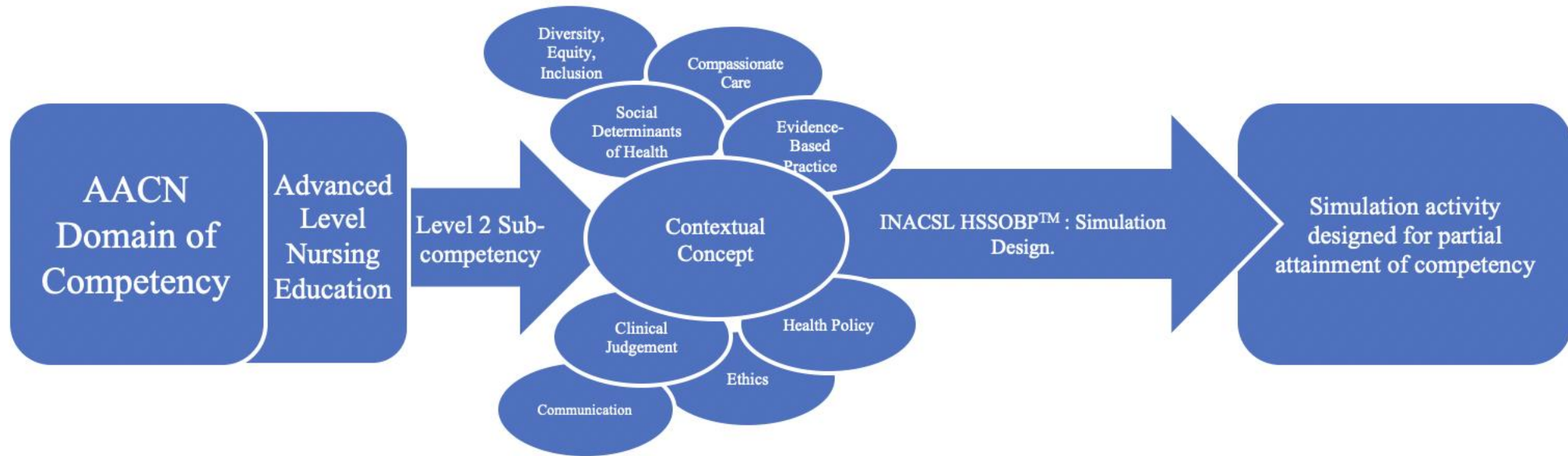
THE ACTS Model

- **Contextual concepts** provide the means through which the competency may be learned, demonstrated, and potentially measured
 - Calls for student conceptualization rather than memorization
 - Grounded in principles of CBE
- **Contextual concepts** are a function of a curricular needs assessment, student knowledge and experience, and how progress toward the competency will be measured
 - Identify vs manage a complex diagnosis
 - UTI vs pituitary apoplexy
- The MacGuffin: sets the plot (quest toward competency) in motion
- *Healthcare providers are not widgets in the economic healthcare enterprise*

Competency-Based Education (CBE)



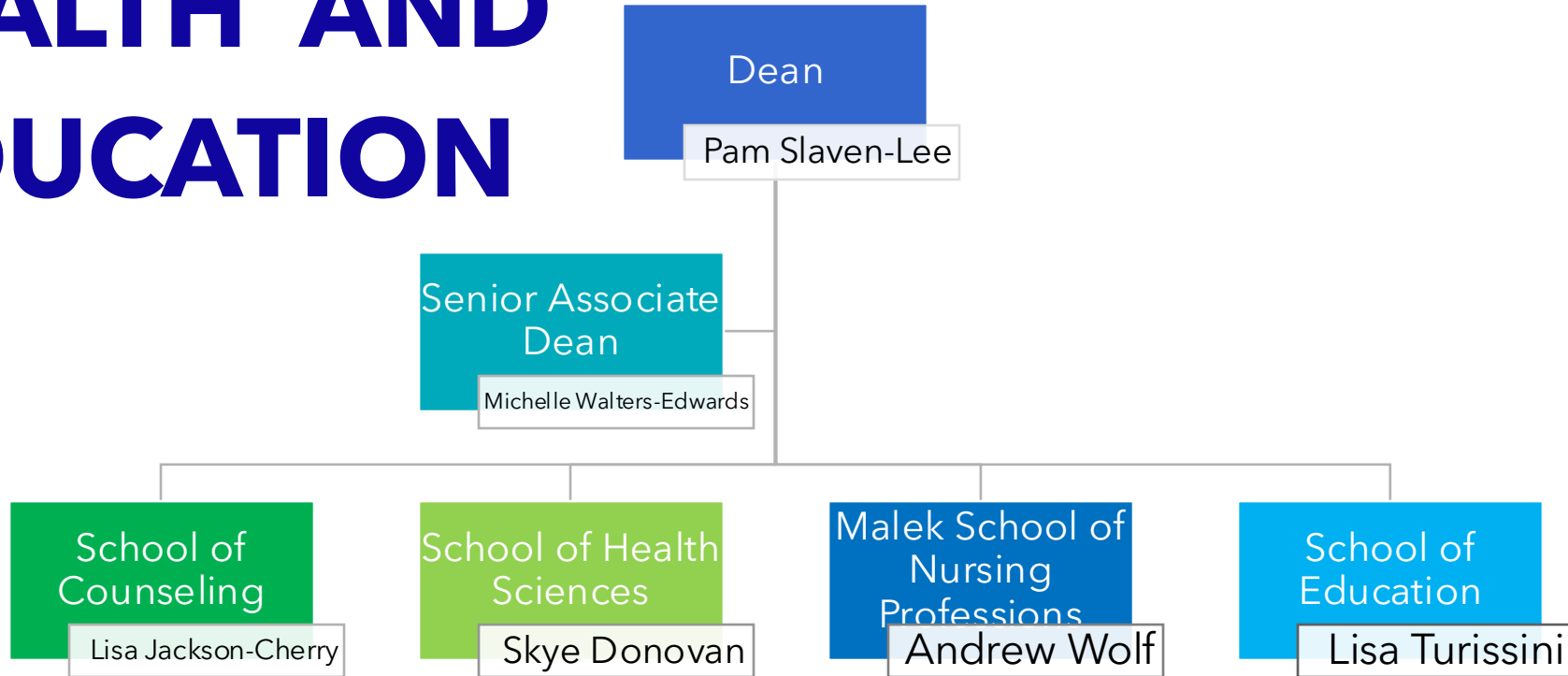
THE ACTS MODEL



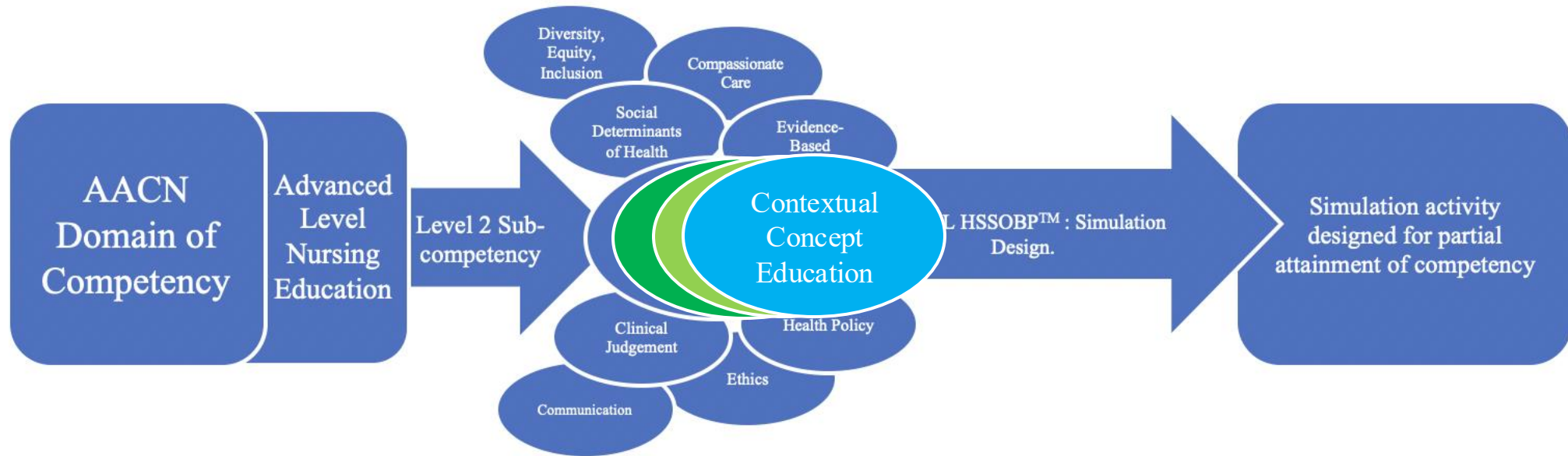
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COLLEGE OF HEALTH AND EDUCATION



THE ACTS MODEL



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INTERDISCIPLINARY AUGMENTATION

PART I: COUNSELING SKILLS AND THERAPEUTIC CONDITIONS

#	Score	Primary Counseling Skill(s)	Specific Counseling Skills and Therapeutic Conditions Descriptors	Exceeds Expectations / Demonstrates Competencies (5)	Meets Expectations / Demonstrates Competencies (4)	Near Expectations / Developing towards Competencies (3)	Below Expectations / Unacceptable (2)	Harmful (1)
1. A		Nonverbal Skills 	Includes Body Position, Eye Contact, Posture, Distance from Client, Voice Tone, Rate of Speech, Use of silence, etc. (<i>attuned to the emotional state and cultural norms of the clients</i>)	Demonstrates effective nonverbal communication skills, conveying connectedness & empathy (85%).	Demonstrates effective nonverbal communication skills for the majority of counseling sessions (70%).	Demonstrates inconsistency in his or her nonverbal communication skills.	Demonstrates limited nonverbal communication skills.	Demonstrates poor nonverbal communication skills such as ignores client &/or gives judgmental looks.
1. B		Encouragers	Includes Minimal Encouragers & Door Openers such as “Tell me more about...”, “Hmm”	Demonstrates appropriate use of encouragers, which supports development of a therapeutic relationship (85%).	Demonstrates appropriate use of encouragers for the majority of counseling sessions, which supports development of a therapeutic relationship (70%).	Demonstrates inconsistency in his or her use of appropriate encouragers.	Demonstrates limited ability to use appropriate encouragers.	Demonstrates poor ability to use appropriate encouragers, such as using skills in a judgmental manner.
1. C		Questions 	Use of Appropriate Open & Closed Questioning (<i>e.g., avoidance of double questions</i>)	Demonstrates appropriate use of open & close-ended questions, with an emphasis on open-ended question (85%).	Demonstrates appropriate use of open & close-ended questions for the majority of counseling sessions (70%).	Demonstrates inconsistency in using open-ended questions & may use closed questions for prolonged periods.	Demonstrates limited ability to use open-ended questions with restricted effectiveness.	Demonstrates poor ability to use open-ended questions, such as questions tend to confuse clients or restrict the counseling process.
1. D		Reflecting ^a Paraphrasing	Basic Reflection of Content – Paraphrasing (<i>With couples and families, paraphrasing the different clients’ multiple perspectives</i>)	Demonstrates appropriate use of paraphrasing as a primary therapeutic approach (85%).	Demonstrates appropriate use of paraphrasing (majority of counseling sessions; 70%).	Demonstrates paraphrasing inconsistently & inaccurately or mechanical or parroted responses.	Demonstrates limited proficiency in paraphrasing or is often inaccurate.	Demonstrates poor ability to paraphrase, such as being judgmental &/or dismissive.
1. E		Reflecting ^b Reflection of Feelings	Reflection of Feelings (<i>With couples and families, reflection of each clients’ feelings</i>)	Demonstrates appropriate use of reflection of feelings as a primary approach (85%).	Demonstrates appropriate use of reflection of feelings (majority of counseling sessions; 70%).	Demonstrates reflection of feelings inconsistently & is <i>not</i> matching the client.	Demonstrates limited proficiency in reflecting feelings &/or is often inaccurate.	Demonstrates poor ability to reflective feelings, such as being judgmental &/or dismissive.



Council for
Accreditation
of Counseling
and Related
Educational
Programs



INTERDISCIPLINARY AUGMENTATION

PART II: COUNSELING DISPOSITIONS AND BEHAVIORS

#	Score	Primary Counseling Dispositions & Behaviors	Specific Counseling Disposition & Behavior Descriptors	Exceeds Expectations / Demonstrates Competencies (5)	Meets Expectations / Demonstrates Competencies (4)	Near Expectations / Developing towards Competencies (3)	Below Expectations / Unacceptable (2)	Harmful (1)
2. A		Professional Ethics 	Adheres to the ethical guidelines of the ACA, ASCA, IAMFC, APA, & NBCC; including practices within competencies	Demonstrates consistent & advanced (<i>i.e., exploration & deliberation</i>) ethical behavior & judgments.	Demonstrates consistent ethical behavior & judgments.	Demonstrates ethical behavior & judgments, but on a concrete level with a basic ethical decision-making process.	Demonstrates limited ethical behavior & judgment, and a limited ethical decision-making process.	Demonstrates poor ethical behavior & judgment, such as violating the ethical codes &/or makes poor decisions
2. B		Professional Behavior 	Behaves in a professional manner towards supervisors, peers, & clients (e.g., emotional regulation); Is respectful and appreciative to the culture of colleagues and is able to effectively collaborate with others	Demonstrates consistent & advanced respectfulness and thoughtfulness, & appropriate within <i>all</i> professional interactions.	Demonstrates consistent respectfulness and thoughtfulness, & appropriate within <i>all</i> professional interactions.	Demonstrates inconsistent respectfulness and thoughtfulness, & appropriate within professional interactions.	Demonstrates limited respectfulness and thoughtfulness & acts inappropriate within some professional interactions.	Demonstrates poor professional behavior, such as repeatedly being disrespectful of others &/or impedes the professional atmosphere of the counseling setting / course.
2. C		Professional & Personal Boundaries	Maintains appropriate boundaries with supervisors, peers, & clients	Demonstrates consistent & strong appropriate boundaries with supervisors, peers, & clients.	Demonstrates consistent appropriate boundaries with supervisors, peers, & clients.	Demonstrates appropriate boundaries inconsistently with supervisors, peers, & clients.	Demonstrates inappropriate boundaries with supervisors, peers, & clients.	Demonstrates poor boundaries with supervisors, peers, & clients; such as engaging in dual relationships.
2. D		Knowledge & Adherence to Site and Course Policies	Demonstrates an understanding & appreciation for <i>all</i> counseling site and course policies & procedures	Demonstrates consistent adherence to <i>all</i> counseling site and course policies & procedures, including strong attendance and engagement.	Demonstrates adherence to most counseling site and course policies & procedures, including strong attendance and engagement.	Demonstrates inconsistent adherence to counseling site and course policies & procedures, including attendance and engagement.	Demonstrates limited adherence to counseling site and course policies & procedures, including attendance and engagement.	Demonstrates poor adherence to counseling site and course policies, such as failing to adhere to policies after discussing with supervisor / instructor.

APPLICATION OF THE ACTS MODEL

Systematic application of the INACSL Design Standard to
AACN Essentials Competency 2.3 (C 2.3)

Level 2 Advanced-Level Nursing Education Sub-Competency
2.3h (SC 2.3h) in the

Domain 2: Person-Centered Care (DPCC)

Learner level: entry into clinical courses

***2.3h Demonstrate that one's practice is informed by a
comprehensive assessment appropriate to the functional
area of advanced nursing practice.***



STEP 1: IDENTIFY DOMIAN, COMPETENCY, CONTEXTUAL CONCEPT

Example: After review of the DNP program Family Nurse Practitioner (FNP) track curriculum map, NP educators determine the need to provide students with an experiential learning opportunity to assess a competency within the patient care domain (C2.3, SC 2.3h, in DPCC).

- Programmatic assessment data suggest that by week 6 of the first of three FNP clinical courses, students acquire knowledge regarding the assessment of an uncomplicated urinary tract

STEP 2: ALIGN OBJECTIVES WITH PROGRESSION/PERFORMANCE INDICATORS

Example: The following measurable objectives are developed in consultation with the simulation expert consistent with level 2 sub-competencies:

1. Elicits pertinent positive and negative subjective data in adult patient presenting with UTI in primary care setting
2. Performs focused physical examination in adult patient presenting with UTI in primary care setting
3. Accurately and succinctly documents subjective and objective assessment findings using history and physical examination template
4. *Demonstrates behaviors consistent with concepts for nursing practice*

STEP 3: SELECT FORMAT TO TEACH, TEST, OR BOTH

Example: For the purpose of teaching and learning, a **formative** case scenario design is selected. The simulation activity is **integrated** into the curriculum at week 7 of the first of three FNP clinical courses after UTI content is covered.

- The simulation activity will require each participant to assess the patient presenting with UTI in the primary care setting and to document their assessment findings using a history and physical examination template.

STEP 4: WRITE CASE TO PROVIDE CONTEXT

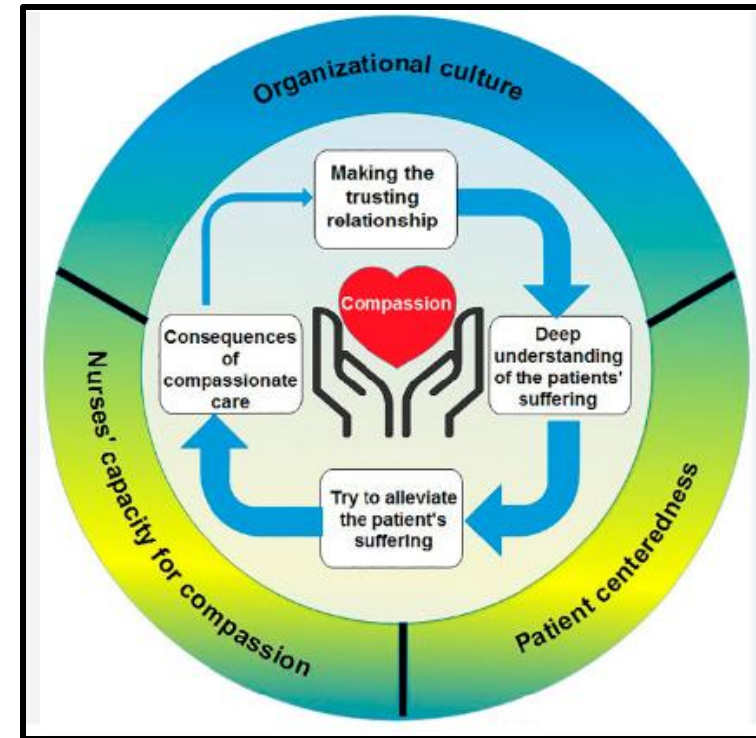
Example: The Association of Standardized Patient Educators Case Development Template is used to develop the case scenario narrative to include the presentation and logical progression of a patient presenting to the primary care clinic with UTI.

- Several elements of the review of systems (ROS) and select physical examination maneuvers are identified as critical actions (e.g., fever, costovertebral tenderness, vaginal discharge, hematuria) and performance measures.
- They are linked to cues, are embedded into the case narrative, and are used to develop a case-specific skills performance assessment inventory

STEP 4: WRITE CASE TO ALLOW FOR EXPERIENTIAL LEARNING OF NURSING CONCEPTS AND FIDELITY

AACN CONCEPTS FOR NURSING PRACTICE

1. Clinical Judgement
2. Communication
3. Compassionate Care
4. Diversity, Equity, and Inclusion
5. Ethics
6. Evidence-Based Practice
7. Health Policy
8. Social Determinants of Health



STEP 5: CREATE PERCEPTION OF REALISM

Example: The simulation activity takes place in a room equipped to replicate a primary care examination room.

- Each participant's encounter with the patient will be limited to 20 minutes to replicate the time typically allotted for a primary care office visit for an episodic complaint such as UTI.
- The case scenario and associated assessment inventories are reviewed by an NP with expertise in assessment and management of UTI.
- The case scenario and checklists are carefully reviewed with the patient and facilitator to ensure desired patient responses and behaviors are consistent with the case scenario narrative.

STEP 6: FACILITATE STUDENT-CENTERED LEARNING

Example: Working in collaboration with the simulationist, all facilitators attend a training session on simulation facilitation during which the principles of student-centered learning are reviewed.

- Facilitators are provided evidence-based resources specific to the assessment of UTI and are prepared to provide the guidance and mentorship necessary for learners to meet the objectives.
- The lead faculty facilitator assumes responsibility for providing the resources necessary to maintain a student-centered approach to facilitation.
- Facilitators are evaluated by the participants and lead faculty facilitator.

STEP 7: PREBRIEF FOR STUDENT SUCCESS

Example: Several weeks prior to the simulation event, facilitators and participants are able to access a recorded prebrief.

- Prebrief includes review of all objectives and expectations of debrief and feedback sessions.
- One week in advance of the event, participants are invited to a webinar in which the materials are presented by the lead faculty facilitator. At that time, they are invited to ask questions.
- The webinar is also recorded and made available for review.
- Immediately prior to the simulation activity, prebrief materials are again reviewed and participants are given an opportunity to tour the simulation space

STEP 8: DEBRIEF AND/OR PROVIDE FEEDBACK

Example: Participants are informed that immediately following the patient encounter there will be a group debrief using the Debrief for Meaningful Learning (DML) model. A DML debriefing prompt is prepared specific to the simulation activity designed to demonstrate C 2.3, SC 2.3h, in the DPCC.

- The script will help ensure standardization of the participant experience by decreasing variability in debriefing style and structure among facilitators (Eppich & Cheng, 2015).



STEP 8: DEBRIEF AND/OR PROVIDE FEEDBACK (CONT.)

- Participants are assured that individual performance will not be critiqued in front of fellow participants; instead, they will be afforded the opportunity to receive feedback on performance relative to the objectives in an individual feedback session.
- Assurance of safe learning environment

(Eppich & Cheng, 2015).



STEP 9: EVALUATE

Example: To provide a formative objective data point reflective of knowledge and skills relative to the concept of UTI, participants are assessed using a skills checklist based on critical actions and performance measures necessary to meet each of the objectives.

- This checklist may be used in the partial attainment of C2.3, SC 2.3h, in the DPCC because UTI serves as the conceptual context (MacGuffin) for the sub-competency.
- Participants are provided an opportunity to complete an anonymous reflective survey regarding the simulation design, the debrief

STEP 9: EVALUATE (CONT.)

- The reflective survey also prompts students to provide information about their perception of whether the preparatory materials were helpful; degree to which they believe the simulation activity improved their ability to assess patients with UTI in the clinical setting; and their progress toward attainment of the competency/sub-competency.
- **All assessment and evaluation data are interpreted with caution given lack of demonstrated validity and reliability.**

Sub-competency: 2.3h Demonstrate that one's practice is informed by a comprehensive assessment appropriate to the functional area of advanced nursing practice.



STEP 10: PREPARATION AND RESOURCES

Example: The simulation will occur after the concept of UTI has been covered in the didactic content and after participants have completed the advanced health assessment course and approximately 100 hours in the adult primary care setting.

- Simulation facilitators create an electronically accessible folder containing preparatory materials including a recording of the prebrief session (or preparatory meeting with the students).
- Although the case-specific performance inventory is not shared with students, the expectations regarding ROS and musculoskeletal examination in a patient presenting with UTI are detailed in the recorded prebrief.
- ***Include review of concepts for nursing care in prebrief***

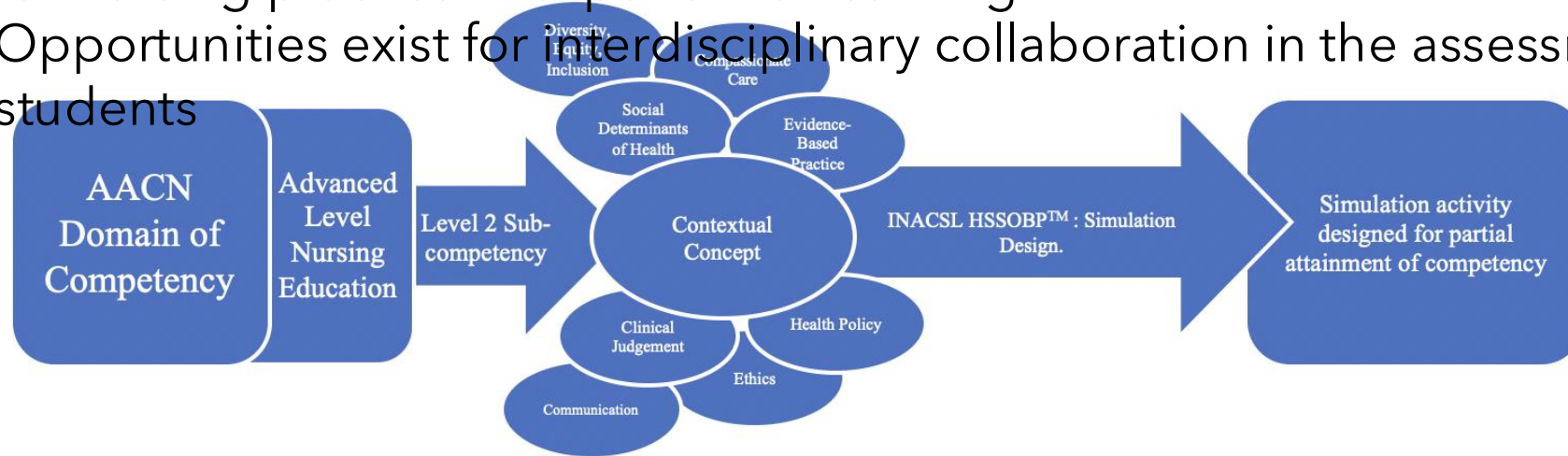
STEP 11: PILOT TEST

Example: A pilot test involving NP students who have completed approximately 100 clinical hours in the adult primary care setting is arranged.

- Faculty that will be facilitating the simulation will participate in the pilot, as will an SP, the SP educator, and the director of the simulation facility.
- The lead faculty facilitator and director of the simulation facility create and communicate a plan to address any issues that might arise during the simulation activity.

SUMMARY

- Incumbent on educators to design a means for students to progress toward attainment of AACN Competencies and sub-competencies
- Application of INACSL SHHOBP™ is essential to optimal student learning
- Thoughtful and systematic curricular integration of simulation is essential to CBE
- The ACTS Model provides a conceptual pathway for the application of INACSL SHHOBP™ to the AACN New Essentials and intentional inclusion of concepts for nursing practice in experiential learning
- Opportunities exist for interdisciplinary collaboration in the assessment of students





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